



2024-25

ANNUAL EVALUATION REPORT

Dependency Advocacy Center, First Call Program



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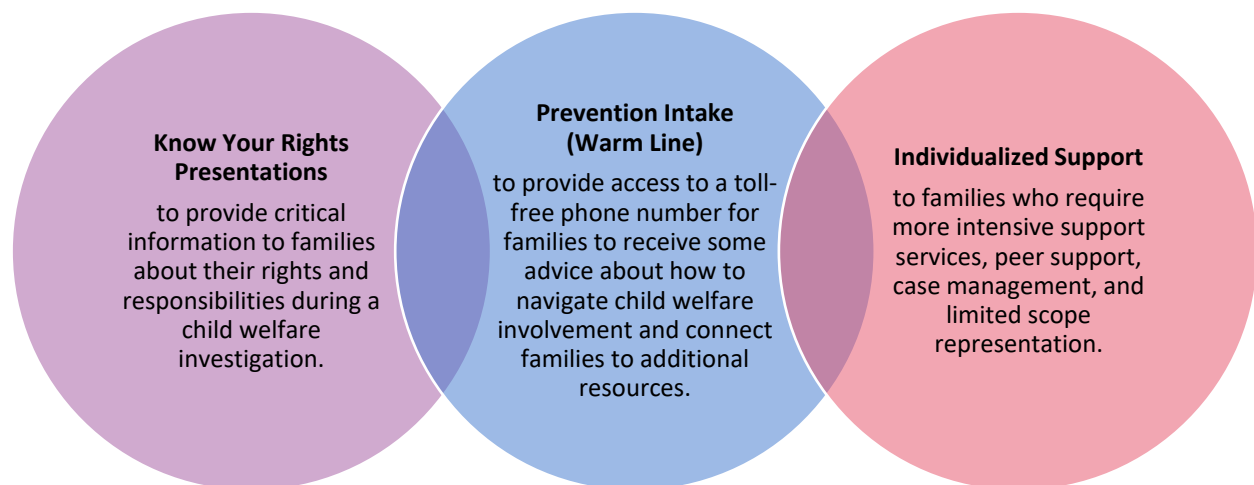
Executive Summary

Dependency Advocacy Center (DAC) is a nonprofit organization that provides legal services to economically disadvantaged parents involved with Santa Clara County's child welfare system who face the possibility of child removal due to allegations of abuse and neglect. Families often find themselves in this predicament due to experiencing symptoms of poverty, trauma, illicit substance use, domestic violence, and mental health struggles. DAC has implemented innovative programs to serve families in a more comprehensive manner by employing in-house social workers and mentor parents and offering legal support to secure clients' legal rights while simultaneously engaging clients to improve their circumstances with an aim to enhance family safety and stability.

Dependency Advocacy Center's Mission

Dependency Advocacy Center provides zealous legal representation to indigent clients in the juvenile dependency system to promote timely reunification and preservation of families in a safe, healthy environment. DAC believes that every parent and child entering the dependency system has a right to be treated with dignity, compassion, and respect.

First Call for Families (First Call) is one of the innovative programs operated by DAC. First Call's goal is to prevent children from being separated from their families by avoiding formal involvement with juvenile dependency court. First Call is made up of three essential components:



The First Call team includes an **attorney** to provide legal advice and support; a **social worker** to provide clinical support services including risk assessments, safety planning, and support during meetings with child welfare social workers; and **mentor parents** to provide hope and support to families as they have experienced first-hand what is it like to be formally involved with the child welfare system and successfully reunified with their children.

First Call launched in 2021 and since September 2022, Santa Clara County's Social Services Agency (SSA) has funded the program. The initiative assists at-risk parents in informed decision-making to promote family stability, decrease disparities in child welfare involvement, and prevent continued system engagement. Partnership with the Department of Family and Children's Services (DFCS) has been crucial to enhancing communication and program outreach among DFCS social workers, particularly among those working with families informally and prior to any juvenile dependency court intervention.¹

¹ While working in partnership with the Department of Family and Children's Services (DFCS), communications between First Call staff and clients are still protected by attorney-client privilege.

KEY FINDINGS FROM FISCAL YEAR 2024-25

Know your Rights Presentations



- First Call for Families held 15 Know Your Rights presentations, reaching a total of 234 participants. Of the 234 participants, 100 were community members and the other 134 were from community-based organizations.
- Over 87% of surveyed participants from the general public and 100% of community-based organizations reported that attending an event increased their knowledge about rights related to the child welfare system.

Families Referred to First Call (Warm Line)



- The Dependency Advocacy Center triaged and connected 223 referrals to the First Call for Families program.
- 193 clients referred to the Dependency Advocacy Center received Initial Consult and Advice services to review communications with DFCS and assess client needs. Among these referrals, 17 cases were assigned to more intensive “Individualized Support” services, and 145 cases were triaged to receive limited counsel and advice only (“Warm Line” only), and 17 were referred to other external resources.
 - Although these families did not receive intensive support services, 100 families report having their needs met in regard to custody, 51 had their needs met regarding navigating the child welfare investigation, 15 felt supported with probate court guardianship, 12 received support for other legal issues
- There were 53 active cases receiving Individualized Support services during FY 2024-25. Top presenting needs included: employment, relapse prevention, and substance use. The greatest needs related to self-sufficiency included employment, substance use, and housing.

Individualized Support



- There were 53 active cases receiving Individualized Support services during FY 2024-25. Top presenting needs for clients who completed an intake during the year (N = 13) included: employment, relapse prevention, and substance use. The greatest needs related to self-sufficiency included employment, substance use, and housing.
- Staff engaged clients in 8 Intake meetings, 751 Interim meetings, and 21 meetings to close-out cases. The most common interim activities included meeting with clients (or communicating by phone, email, or text) and communicating with DFCS social workers
- 28 clients exited services during the fiscal year. Among them, 21 had documented outcomes. At program exit, clients showed the greatest self-sufficiency gains in domains of legal, family/social relations, and substance abuse, with the largest increase occurring in the legal domain.

Introduction

PROJECT BACKGROUND

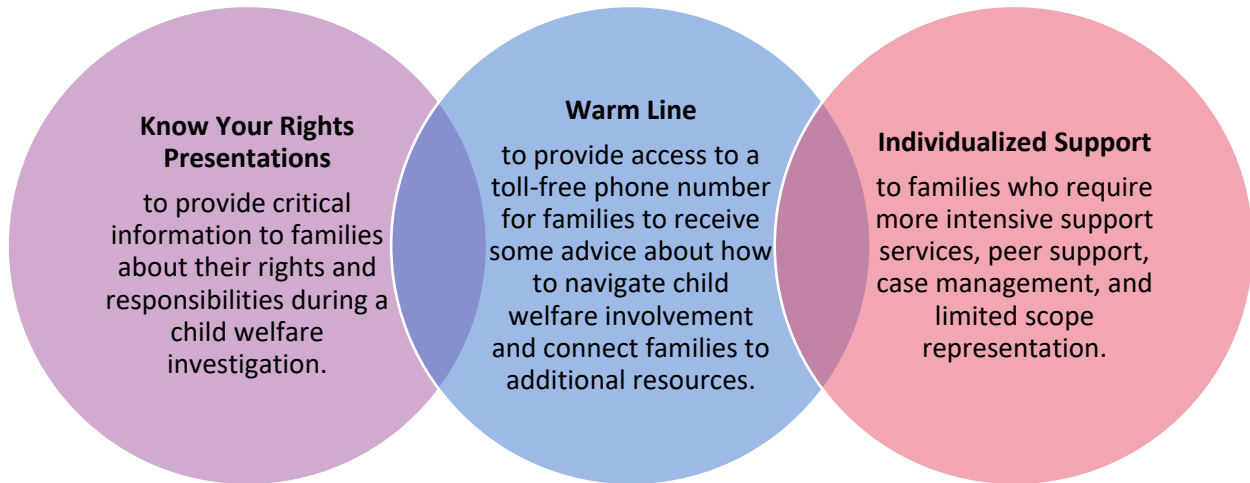
Dependency Advocacy Center (DAC) provides zealous legal representation to indigent clients in the juvenile dependency system to promote timely reunification and preservation of families in a safe, healthy environment. DAC believes that every parent and child entering the dependency system has a right to be treated with dignity, compassion, and respect.

First Call for Families (First Call) is one of the innovative programs operated by DAC. First Call launched in 2021 and since September 2022, Santa Clara County's Social Services Agency (SSA) has funded the program. The initiative assists at-risk parents in informed decision-making to promote family stability, decrease disparities in child welfare involvement, and prevent continued system involvement. Partnership with DFCS has been crucial to enhancing communication and program outreach among DFCS social workers, particularly among those working with families informally and prior to any court intervention.

First Call's goals are to:

- Increase awareness and knowledge of legal rights associated with child welfare system involvement.
- Increase stabilization and self-sufficiency of families who are at risk of becoming formally involved with juvenile dependency court.
- Reduce rates of substantiated abuse and neglect referrals for children whose families are receiving *individualized support*, up to 6 months post-services.
- Reduce rates of sustained petitions for children whose families are receiving *individualized support*, up to 6 months post-services.

First Call is made up of three essential components:



The First Call team includes an **attorney** to provide legal advice and support; a **social worker** to provide clinical support services including risk assessments, safety planning, and support during meetings with child welfare social workers; and **mentor parents** to provide hope and support to families as they have experienced first-hand what is it like to be formally involved with the child welfare system and successfully reunified with their children.

STRATEGIC FRAMEWORK

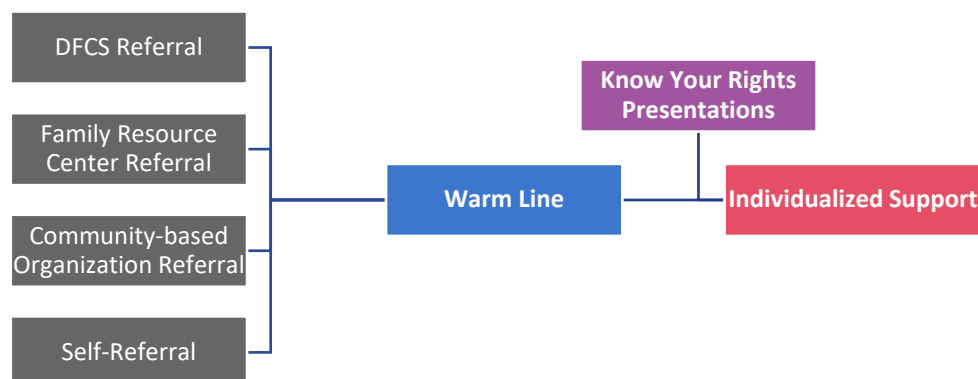
First Call focuses on reaching parents in the areas of the county with disproportionate allegation rates of child maltreatment. First Call expanded its focus on priority zip codes in the last year and includes families living in zip codes 95111, 95112, 95127, 95116, 95020, and 95122 who have been contacted by DFCS within the last 12 months or who are currently receiving informal supervision or non-court DFCS services.

First Call’s goal is to reduce the rate of petitions among such families by implementing a three-tiered approach:

- Creating a **toll-free warm line** to offer advice about how to navigate child welfare involvement and resource and referral information. Families can call in directly to the Warm Line or may be referred by a DFCS social worker, a Family Resource Center, a community-based organization, or a friend/relative.
- Delivering “Know Your Rights” **community presentations** to families to help them learn what rights and responsibilities they have during a DFCS investigation. Families may be referred from the Warm Line or a community-based organization to participate in these presentations.
- Providing intensive targeted case management and representation to parents/caregivers who require more **individualized support**, including ongoing case management, peer mentoring, and/or limited scope representation.

This flow chart depicts the typical entry points for the parents and caregivers who work with First Call. Families may be referred to the **Warm Line**, which then refers them to the **Know Your Rights presentations**, or vice versa. From there, if families need additional support, they will be referred for **Individualized Support**.

Referral and Support Services Flow Chart



DAC partnered with Applied Survey Research (ASR) to conduct an evaluation of the program in fiscal year 2023-24. The evaluation team developed **data collection tools** and **data dashboards to measure implementation and outcomes**. The list below shows the various data collection tools/processes:

DATA COLLECTION TOOLS

Know Your Rights Activity Log	Populated by First Call for Families staff with aggregate data collected to describe participant demographics.
Know Your Rights Surveys	Collects responses for two surveys: Know Your Rights Participation Surveys for Community-Based Organizations and Know Your Rights Participation Surveys for the General Public.
Prevention Intake Form	Populated by DAC staff. Includes demographic information of families served by the program and used to triage clients into relevant DAC programs.
Initial Consult and Advice / Closing Questions	Used to further determine appropriate services for clients assigned to the First Call program at Intake.
Individualized Support Log	Populated by First Call for Families staff. Includes separate records for each engagement.
Client Satisfaction Surveys	Collect responses from clients who exit Individualized Support.
Exit Interviews	Exit interviews were conducted by external evaluation partners at Applied Survey Research to capture details about the success and challenges of the First Call Program.

Highlighted findings from fiscal year 2024-25 evaluation are outlined in the sections below.

Implementation Monitoring

KNOW YOUR RIGHTS PRESENTATIONS

First Call for Families has been delivering “Know Your Rights” community presentations to families to help them learn what rights and responsibilities they have during a DFCS investigation.

First Call for Families hosted 15 Know Your Rights presentations reaching 234 participants.

First Call staff hosted 15 presentations, of which seven presentations were directed to community members from the general public, with a total of 100 participants, and eight presentations were for staff located at CBOs and other partner organizations, with a total of 134 participants. Two of the presentations were co-presented with Dependency Advocacy Center’s Corridor program.



Community members reported increased knowledge and awareness of their legal rights.

Community member attendees were asked to participate in an electronic or paper survey for each Know Your Rights presentation to identify how families found out about the service, whether they were residing in the priority zip codes, and key demographics to ensure that diverse families were supported by this effort. Sixteen community members who attended the Know Your Rights presentations completed the survey (16% completion rate).

Over 87% (N = 14) of the community member attendees reported that participation in the Know Your Rights presentation increased their knowledge and awareness of legal rights associated with child welfare system involvement. Topics included a **general overview of dependency law and process** and **what to do if you get contacted by CPS**.

Community partners reported increased knowledge and awareness of how families are impacted by child welfare involvement.

Twenty-eight community-based organization (CBO) participants completed the survey on the Know Your Rights presentations, and all of them (100%) reported that the presentation they attended

increased their knowledge and/or awareness of the child welfare system. Respondents indicated interest in attending additional trainings.

CBO survey participants found learning about dependency court practices and outcomes particularly useful.

CBO survey participants were also asked what areas of the presentation were useful. They mentioned the presentations gave valuable information on:

- Common practices of social workers during investigations.
- The different types of courts and possible outcomes in CPS cases.
- The data shared was insightful to understand the role of Dependency Advocacy Center in the Child Welfare system.

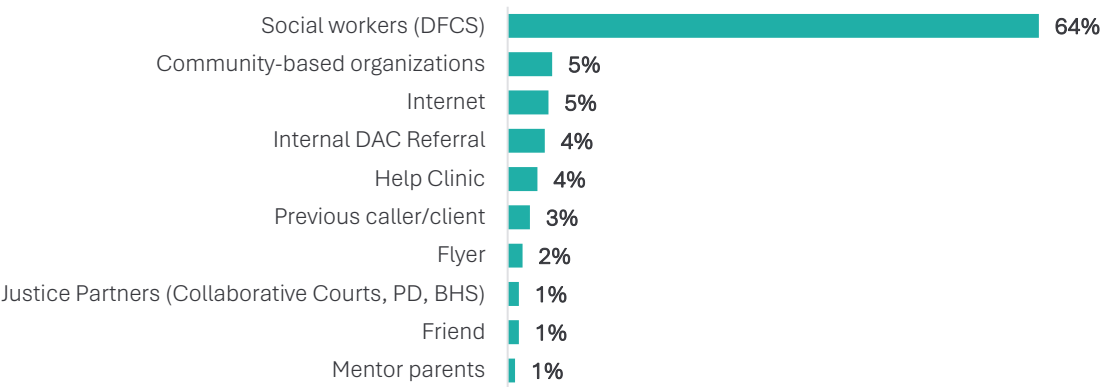
CLIENTS TRIAGED THROUGH A NEW PREVENTION INTAKE PROCESS

The **Prevention Intake Specialist** at the Dependency Advocacy Center screens referrals and assigns cases to two programs supporting families: First Call for Families or the Corridor program. During the 2024-25 fiscal year, the Prevention Specialist processed 223 referrals to the First Call for Families program.

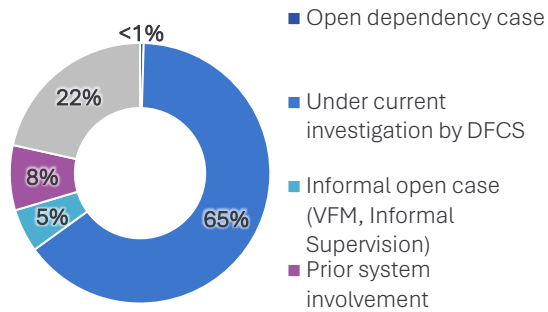
The majority of families were referred to the First Call for Families program through social workers from the child welfare system.

The majority of participants were referred through formal channels, with 143 individuals (64%) being referred by a social worker within the Department of Family and Children’s Services, making this the most common source of referrals among First Call for Families cases. Other referral sources are shown in the figure below.

Figure 1. Referrals Triaged to First Call for Families by Referral Source (N = 223)



Most families engaged with the program had active involvement with child welfare services. Specifically, 144 families (65%) had current investigation with child welfare (only 1 family had an open dependency case). Meanwhile, 12 families (5%) had an informal open case, and another 18 (8%) had prior system involvement. Notably, 48 families (22%) reported having no current or prior contact with the Department of Family and Children’s Services.

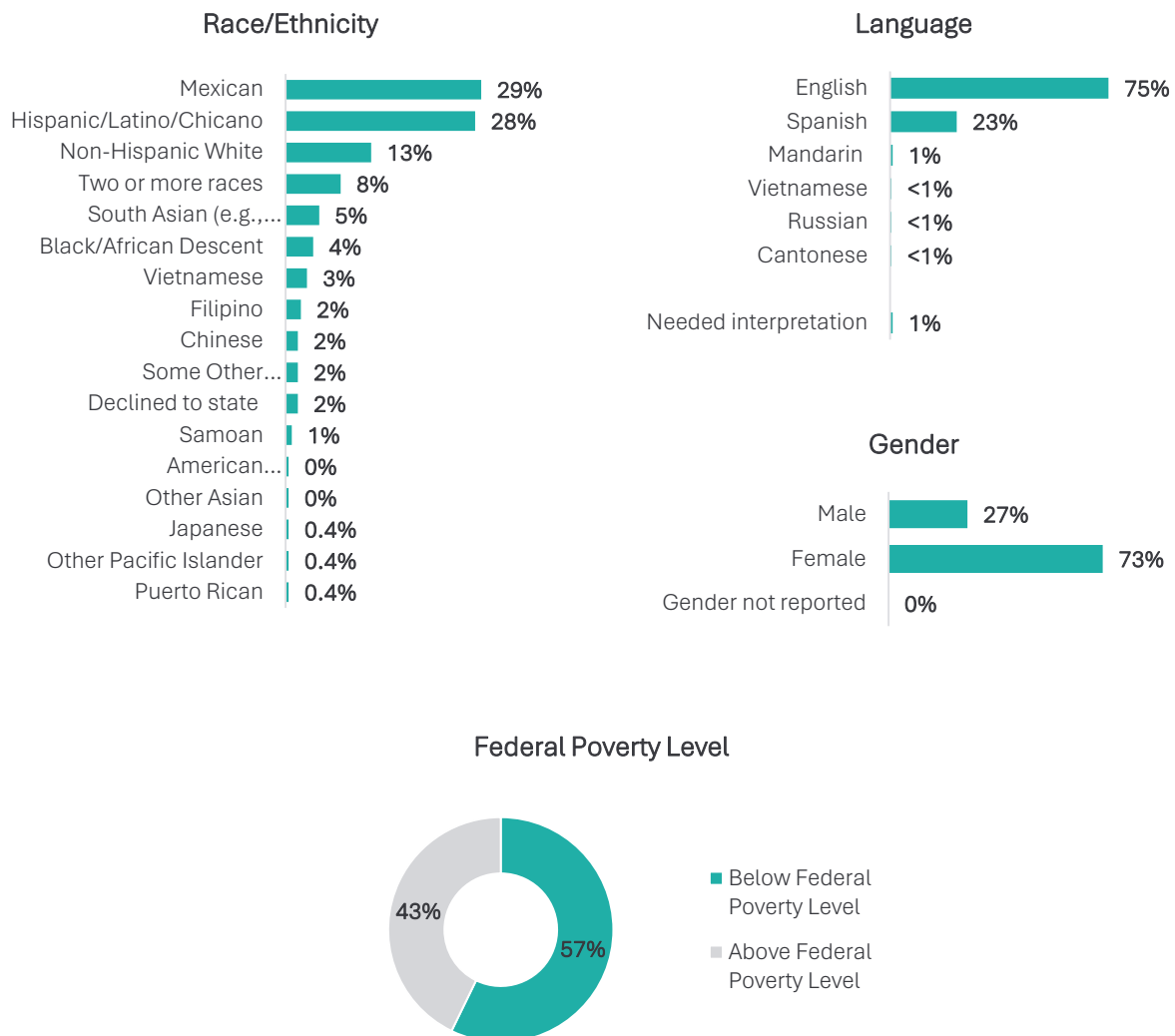
Figure 2. Involvement with Child Welfare Services at Time of Referral (N = 223)

The demographic snapshot highlights need for targeted support among First Call's low-income, multicultural families.

Parents referred to the First Call program ranged in age from 17 to 63 years, with an average age of 38. Across the 223 records, families represented a total of 423 children, averaging two children per household and ranging from one to seven children.

The majority of records were associated with individuals identifying as female (73%), with only a quarter identifying as male (27%). Over half of parents served identified as either Hispanic/Latino/Chicano (28%) or Mexican (29%), and most spoke English as their primary language (75%), followed by Spanish (23%). There were two individuals who required interpretation support. Records were distributed across 48 ZIP codes, with the largest shares from ZIP codes 95112 (5%), 95050 (5%), and 95127 (5%); there were 11% of records with missing ZIP code information. Twelve clients identified themselves as pregnant/expecting, ten of the individuals served identified themselves as having a disability, and none of them identified as a veteran. Just over half (57%) of families served by First Call also earned less than the federal poverty level.

Figure 3. Demographic Characteristics of First Call for Families Case Records, FY 2024-25 (N=223)



The Prevention Specialist conducts a conflicts check for each referral to assess eligibility for First Call for Families services.

Referrals to Dependency Advocacy Center required additional documentation and eligibility review. Among the 223 cases referred to First Call for Families, assessments were conducted to determine each family's level of involvement with child welfare and criminal justice systems. The findings revealed:

- 160 families (71.7%) were involved with **Department of Family and Children's Services**, either through a recent investigation or by receiving informal or voluntary services.
- 4 families (1.8%) had an **open dependency court case** in Santa Clara County.

- 30 families (13.5%) had a **history of dependency court involvement** in the county.
- 63 families (28.3%) had **current or prior criminal justice involvement**.
- 13 families (5.8%) were **on probation** in Santa Clara County.
- 4 families (1.8%) were receiving **pretrial services**.
- 26 families (11.7%) were the subject of a **restraining order**.
- 7 families (3.1%) were under a **criminal protective order (CPO)**, where the restrained party was the client and the protected party was their child.

The remainder of this report will focus on the services and data compiled by the First Call for Families team for all services rendered.

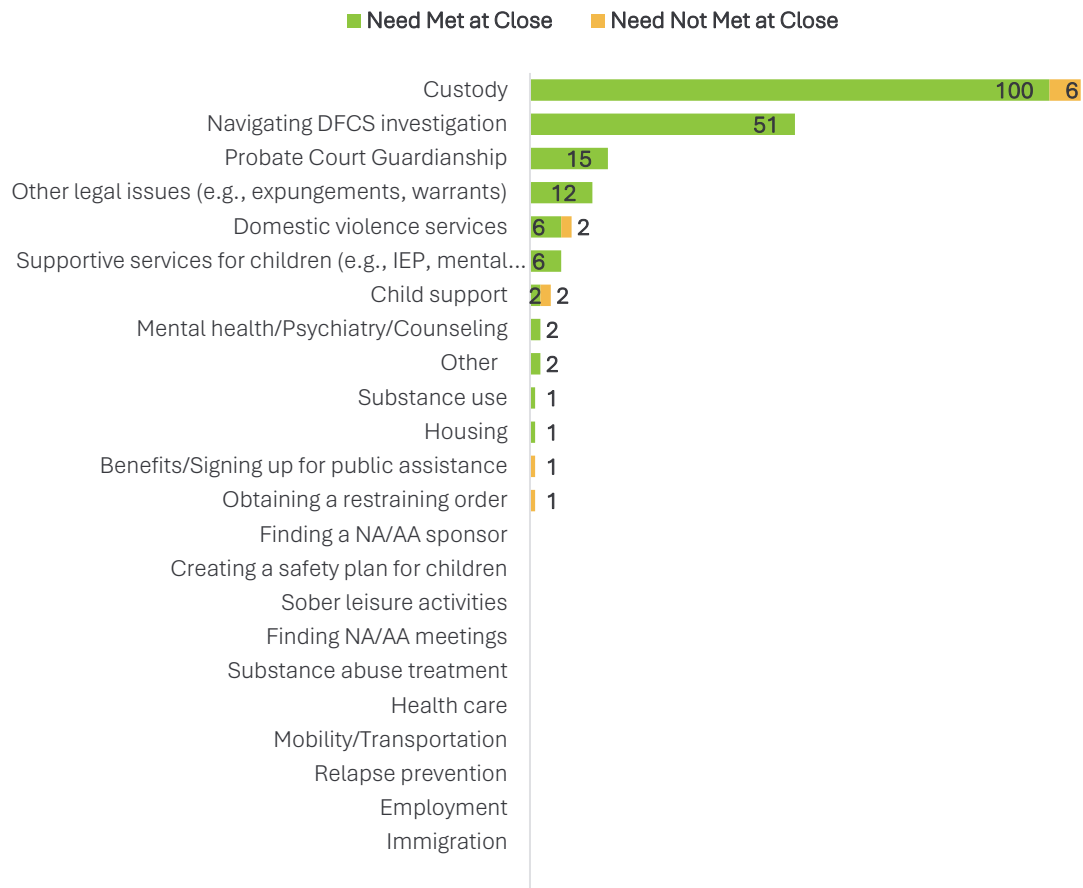
FIRST CALL FOR FAMILIES INITIAL CONSULT AND ADVICE

For all referrals assigned to the First Call for Families program, the First Call team initiates an initial consult and advice session to ensure families are connected with the most appropriate services. During FY 2024-25, a total of **193 clients were served through this process** (30 clients did not have completed Intake at the time the fiscal year ended). This process includes documenting communications with the client, Department of Family and Children's Services, as well as reviewing relevant documents and assessing client needs. Clients are then triaged (First Call Closing Questions) into more intensive services through Individualized Support or otherwise referred to other resources.

- 145 clients received limited counsel and advice only
- 13 clients were referred to external resources (i.e., other legal organizations, Family Court Self-help Center)
- 17 clients were referred to Individualized Support for more intensive supportive services

In some cases, the team may document that the client was unable to be contacted (9), or that a conflict of interest prevents further involvement (9).

Families who received limited consult and advice services only (N = 145), or who are referred to external resources (N = 13), are asked to share information about how the First Call for Families program met their needs. Although these families did not receive intensive support services, 100 families report having their needs met in regard to custody, 51 had their needs met regarding navigating the child welfare investigation, 15 felt supported with probate court guardianship, 12 received support for other legal issues, 6 received help with domestic violence services (see figure below).

Figure 4. Needs and assets of clients served in Individualized Support at Intake (N = 158)

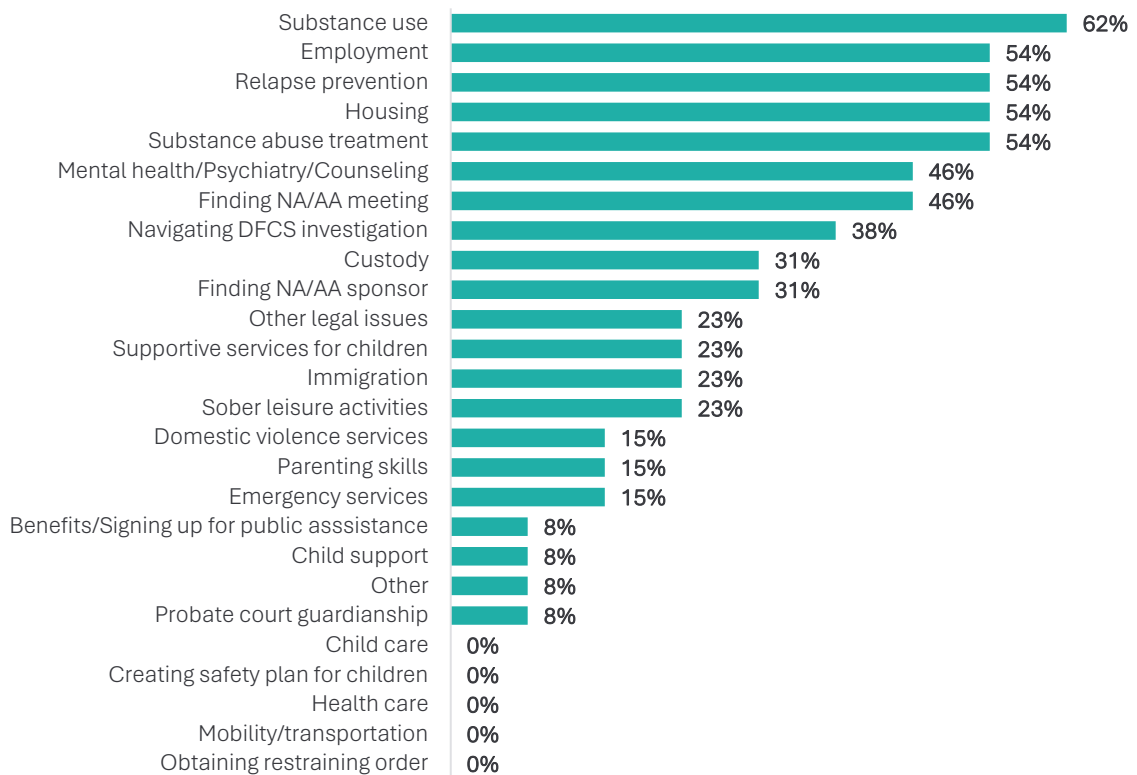
INDIVIDUALIZED SUPPORT

Families with more intensive case management and representation needs were referred from the Warm Line to Individualized Support. First Call supported parents/caregivers who required more individualized support, including ongoing case management, peer mentoring, and/or limited scope representation.

There were 53 families who received intensive services through Individualized Support.

In FY 2024-25, there were 53 cases active in Individualized Support services. There were 8 families that had an intake meeting and 28 who exited services during FY 2024-25 of which 21 of them completed the program. Family needs were documented at intake to be able to customize support and address unmet needs (see figure below). The most common needs among families receiving Individualized Support were for **employment, substance use, and relapse prevention**.

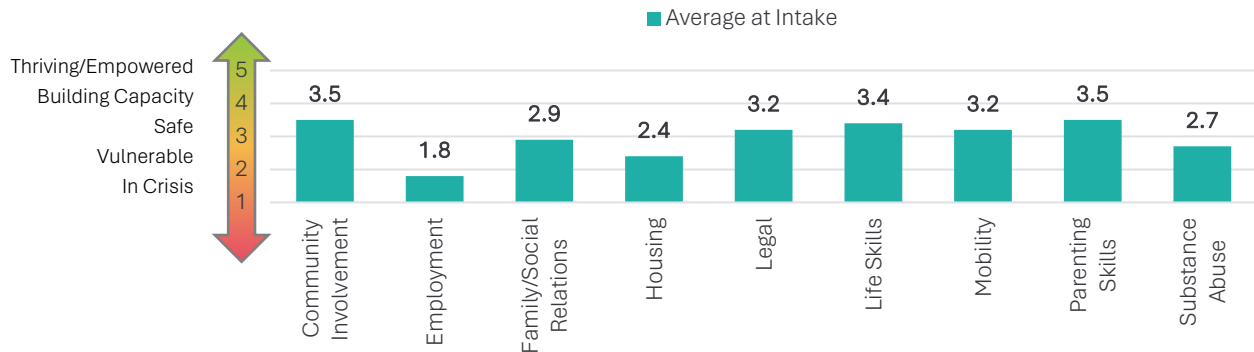
Figure 5. Needs of families served in Individualized Support at Intake (N = 13)



Self-sufficiency was evaluated at intake for all clients. First Call social workers and mentor parents utilize the Self-Sufficiency Matrix to rate the client's status in nine domains: housing, employment, mobility, life skills, family relationships, community involvement, parenting skills, legal issues, and substance use. Domains are scored to identify families who are (1) in crisis, (2) vulnerable, (3) safe, (4) building capacity, or (5) thriving/empowered. Among families with an intake documented during FY

2024-25, the areas with the lowest self-sufficiency scores were for employment, housing, and substance use (average score below “safe” range).

Figure 6. Average Self-Sufficiency Scores at Intake (N = 12)



In response to the identified needs of the family, First Call for Families staff provided ongoing support and case management by meeting with clients and DFCS social workers.

Staff engaged clients in 8 Intake meetings, 751 Interim meetings, and 21 meetings to close-out cases. Interim meeting activities are detailed in the figure below. The most common activities included meeting with clients (or communicating by phone, email, or text) and communicating with DFCS social workers.

Figure 7. Interim meeting activities (751 meetings)

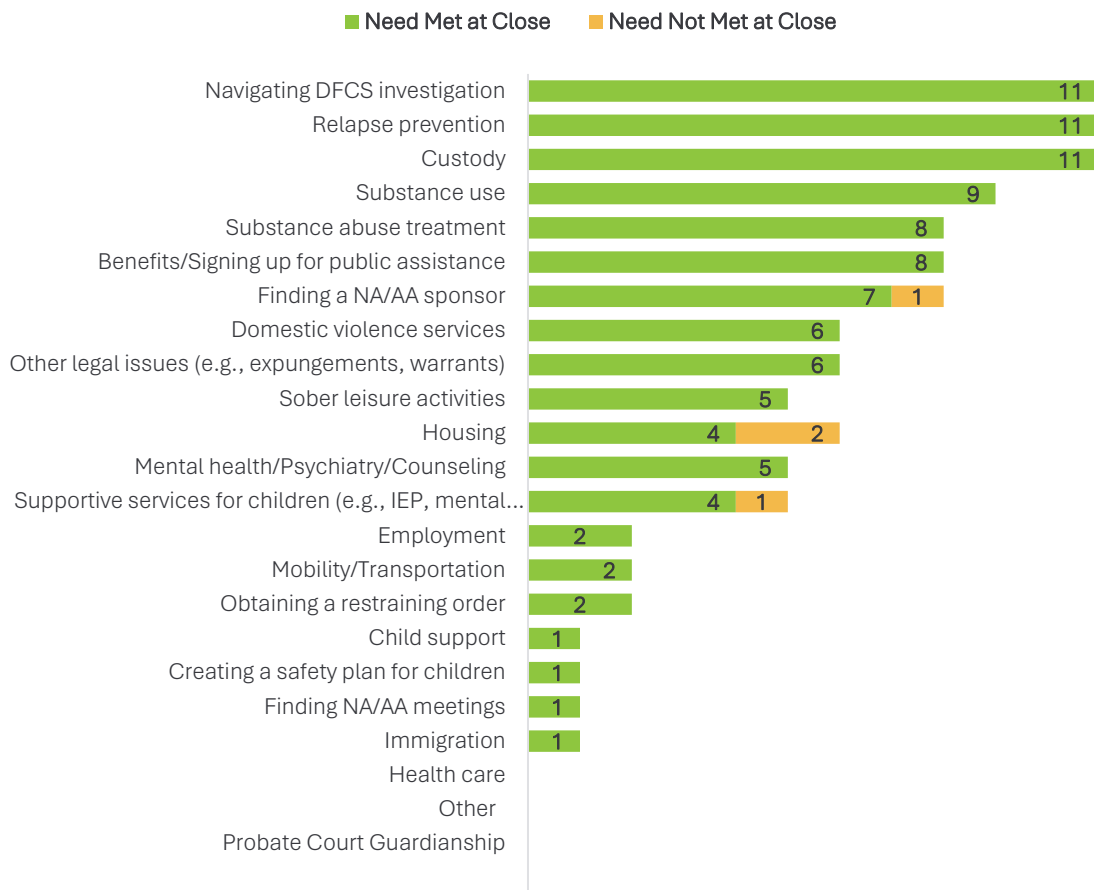
Interim Meeting Activities	N	%
Communication		86%
Texted client	361	48%
Called client	304	40%
Met with client in person	168	22%
Communicated with DFCS social worker	61	8%
Communicated with other community-based organization	28	4%
Sent email / letter to client	24	3%
Case management	41	5%
Participated in Child Family Team meetings (DFCS)	34	5%
Attended court hearing	23	3%
Developed safety plan	2	<1%
Prepared legal documents	3	<1%

Twenty-one Individualized Support cases were closed out by the end of the fiscal year with documented outcomes.

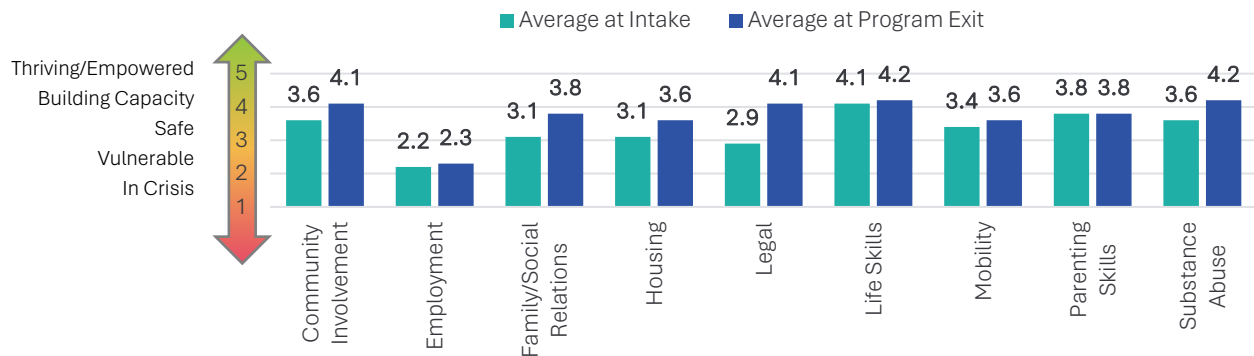
Among the 53 cases, 21 completed program services and have documented outcomes and 7 clients exited services without a formal case closure. This evaluation report includes a summary of key outcomes for families with a documented case closure (excluding no-shows). Outcomes for this evaluation include the percentage of families who had their needs met by case closure, the changes in self-sufficiency from intake to case closure, and the placement and stability of families served.

At case closure, the First Call staff document the needs of each client served through the Individualized Support Services that have or have not been addressed at the time the family exits the program (see figure below).

Figure 8. Needs met among families exiting Individualized Support (N = 21)



Self-sufficiency was also re-evaluated when clients exited the program. Clients demonstrated **increased self-sufficiency in the domains of legal, family/social relations, and substance abuse**, with the largest increase occurring in the legal domain. Self-sufficiency decreased remained relatively stable in the parenting skills, employment, and life skills domains.

Figure 9. Average Self-Sufficiency Scores for Support Clients with Outcomes (N = 17)

At the time that families exit the program, First Call staff also document the placement of children to identify any changes that took place since intake. Placement information was documented for 21 families who completed the program. For 19 of these families, the child's placement was with the parent/guardian at program exit and two were placed in foster care after a petition was filed with the court.

Only 2 out of 21 families who had exited First Call services had an open dependency case with the Department of Family and Children Services.

Among the 21 clients who completed program services, 6 of them had no contact with the Department of Family and Children's Services (DFCS) at the time of program completion, 13 had been contacted by a CPS social worker in the past 6-12 months but did not have an open case, and 2 had an open case.

DFCS evaluates family progress six months after completing First Call services. For families who received at least 45 days of services, DFCS tracks whether they had new contact with the system within six months of exiting.

Of the 25 families who met this criterion, DFCS identified outcomes for 22:

- **15 families** had no new contact with the system.
- **7 families** had some type of new contact:
 - 1 family had multiple referrals
 - 2 families had a sustained petition
 - 4 families were evaluated out
 - 3 families had unsubstantiated allegations (2 unfounded, 1 inconclusive)
 - None of the families had a substantiated allegation

Summary of Client Exit Interview Findings

Applied Survey Research conducted exit interviews on behalf of DAC to learn more about participant experiences in the program within 30 days of program exit. Program exit was determined by the date the attorney documented the case closed. Participants were asked about their overall experience in the program, how it impacted their lives, and suggestions for improvement. In total, two interviews were conducted, each lasting approximately 20 to 30 minutes.

OVERALL EXPERIENCE IN THE PROGRAM

Overall, participants described First Call for Families as a source of consistent, compassionate support that exceeded expectations and helped them regain stability and control over their lives.

Participants shared overwhelmingly positive experiences, highlighting the program's role in helping them navigate recovery, legal concerns, and parenting challenges. They emphasized that staff provided ongoing emotional support and practical assistance — even after their cases had closed — and helped them feel empowered and hopeful about their futures.

“The program helped me get an action plan for getting into recovery so I wouldn’t be in danger of losing custody of my child. They supported me in early recovery. The biggest thing that they contributed was recovery and peer support.”

Program staff were credited with helping participants feel heard and capable. Participants appreciated the support in managing logistics like court appointments, transportation, and navigating systems such as the DMV. Staff were described as reliable, responsive, and committed to staying connected throughout participants' journeys.

“It [First Call] was amazing. It helped me so much through my journey of becoming a mom and staying sober. They helped me with court dates, transportation, legal issues — even came with me for my driving test to get my license. Even with my case closed, we are still in touch and they take my questions.”

Participants also reported feeling more empowered and stable after participating in the program, citing staff assistance with recovery, job support, and access to housing and sober living programs.

Program participants noted staff support in areas such as:

- Developing a recovery plan and accessing peer support

- Navigating legal and court-related issues
- Finding transportation to key appointments
- Accessing housing and job opportunities
- Feeling empowered to take next steps in life

“All the time they were sending me links about new opportunities for housing or job programs. They provided information on sober living programs.”

OPPORTUNITIES FOR IMPROVEMENT

One participant suggested adding a peer support component to help clients connect with others in similar situations.

While both participants praised the First Call for Families program and said they would recommend it to others, one participant mentioned that the experience could be enhanced by facilitating opportunities for peer connection among clients. This feedback suggests the potential value of a peer support group or community-building space within the program structure.

“It would have been nice to have met other people working with First Call, so women working with First Call could have met each other and supported each other. Like a peer support group of individuals in the program.”

The second participant did not identify any changes and emphasized satisfaction with the program’s current approach. Both participants said they had already recommended First call for Families to others in similar situations.

“Just to stay the way they are. It’s amazing.”

FINAL THOUGHTS

Participants who were exited from the First Call for Families program and completed an exit interview were very positive about their experience in the program. They felt the program helped them to navigate the legal process, supported them and their family through a difficult time, provided strong advocacy efforts, and provided key resources to participants when needed. Participants felt so strongly about the positive impact of the program that they have already recommended the program to others they know including family and friends. They appreciated the program being available whenever questions arose during the process and the support to navigate a variety of legal situations.

Participants who completed an exit interview after finishing the First Call for Families program expressed deep appreciation for the support they received. They emphasized the compassion and reliability of program staff, describing how the program made a meaningful difference during a

vulnerable time in their lives. Participants noted the emotional and practical support they received—from help during hospital visits to simply having people they could count on.

“I truly appreciate everything they did for me. They were there at the hospital when my baby was born. They are just really great people, and I am so grateful.”

“It was just amazing. It made my life so much better. I had so much support. I had so many people I could count on. For me and my baby — it was just amazing.”

Both participants described the program as life-changing and indicated they had already recommended it to others. Their reflections speak to the strength of the relationships built with program staff and the lasting impact of the support they received.

Summary of Client Satisfaction Surveys

Clients exiting Individualized Support were asked to participate in a Client Satisfaction Survey to gain deeper insights into their experiences with services received from First Call. Eleven surveys were completed during the FY 2024-25. As displayed in the figure below, respondents strongly endorsed the services provided by First Call and expressed high levels of satisfaction with their experiences. There appears to be only one client who responded “Strongly disagree” to these survey items.

Figure 10. First Call Client Satisfaction Survey Results (N = 11)



The First Call team provided guidance, resources, and encouragement when families needed it most.

The First Call team was described as highly supportive, responsive, and resourceful—helping participants with both emotional needs and concrete tasks, while making a meaningful difference in their recovery and family stability.

- **Accessibility & Responsiveness:** Participants emphasized that the team was always available by phone/text and quick to respond.
- **Resource Connection:** Many highlighted being linked to needed resources, including treatment programs, job opportunities, and county services.
- **Emotional Support & Reassurance:** Respondents valued encouragement, mentorship, and having someone by their side during stressful times.
- **Practical Assistance:** Help with creating safety plans, navigating family court, making appointments, attending meetings (CFTs, outside agencies), and providing clear explanations of processes.
- **Life Impact:** A few noted that the support gave them hope, faith, and motivation—one specifically credited the team with helping them stop substance use.
- **Ongoing Presence:** Several appreciated the team’s continued support across their journey, including home visits and checking in on mental health.

Families expressed overwhelming appreciation, describing First Call as life-changing, supportive, and deeply valued. The only tangible feedback was the idea of expanding **peer mentorship** opportunities, allowing former participants to support other parents. When asked about additional services that could help families in the child welfare system, many again had no suggestions, but some highlighted the need for **practical support** (bus passes, insurance information, job search assistance), **family support groups**, and **more domestic violence resources**.

Looking Forward

Partnership with DFCS and ASR to Continuously Improve

DAC's First Call program will continue to partner with DFCS to support families, and will collaborate with ASR to refine the evaluation plan and data collection tools, with the goal of tracking and reporting outcomes across both DAC prevention programs (First Call and Corridor).

First Call's implementation was documented using Google Forms and Sheets, with program outputs and outcomes visualized in a Google Looker Studio Dashboard for internal monitoring. Throughout implementation, ASR has supported the development and refinement of these tools. Following recent updates to unify data collection between First Call and Corridor, ASR will provide ongoing technical assistance to ensure data accuracy and minimize the burden on staff.

Appendix 1 — Self-Sufficiency Matrix

Client Name: _____ Date: _____

Adapted Self-Sufficiency Matrix (Completed by Mentor Parent or Social Worker)

PART 1: PAST STATUS

Please circle the number that best indicated client status **BEFORE STARTING services with Corridor** using the scale from 1 to 5 (from 1=in crisis to 5 = thriving/empowered)

Domain	1 (in crisis)	2 (vulnerable)	3 (safe)	4 (building capacity)	5 (thriving/ empowered)	Unable to Assess
Housing	1	2	3	4	5	UA
Employment	1	2	3	4	5	UA
Mobility (transportation)	1	2	3	4	5	UA
Life Skills	1	2	3	4	5	UA
Family /Social Relations	1	2	3	4	5	UA
Community Involvement	1	2	3	4	5	UA
Parenting Skills	1	2	3	4	5	UA
Legal	1	2	3	4	5	UA
Substance Abuse	1	2	3	4	5	UA

PART 2: PRESENT STATUS

Please circle the number that best indicates client current status **AFTER services with Corridor** using the scale from 1 to 5 (from 1=in crisis to 5 = thriving/empowered)

Domain	1 (in crisis)	2 (vulnerable)	3 (safe)	4 (building capacity)	5 (thriving/ empowered)	Unable to Assess
Housing	1	2	3	4	5	UA
Employment	1	2	3	4	5	UA
Mobility (transportation)	1	2	3	4	5	UA
Life Skills	1	2	3	4	5	UA
Family /Social Relations	1	2	3	4	5	UA
Community Involvement	1	2	3	4	5	UA
Parenting Skills	1	2	3	4	5	UA
Legal	1	2	3	4	5	UA
Substance Abuse	1	2	3	4	5	UA

Completed by: _____ (MP or SW name)

Administered at approximately _____ months of services:

REFERENCE SHEET: Definitions for rating self-sufficiency in each domain

Domain	1 (in crisis)	2 (vulnerable)	3 (safe)	4 (building capacity)	5 (thriving/empowered)
Housing	Homeless [use if client is incarcerated]	- In transitional or temporary housing - Housing payment is unaffordable	In stable housing that is safe, but is not adequate	In stable subsidized housing that is safe and adequate.	Unsubsidized household is safe and adequate.
Employment	No job. [use if client is incarcerated]	- Temporary - Part-time - Seasonal - Not enough pay to live	- Employed full time - Inadequate pay to live - Few or no benefits.	- Employed full time - Enough pay to live - Benefits.	Maintains permanent employment.
Mobility	No access to transportation, public or private; may have car that is inoperable. [use if client is incarcerated]	Transportation is available, but unreliable, unpredictable, unaffordable; may have car but no insurance, license, etc.	Transportation is available and reliable, but limited and/or inconvenient; drivers are licensed and minimally insured.	Transportation is generally accessible to meet basic travel needs.	Transportation is readily available and affordable; car is adequately insured
Life Skills	Unable to meet basic needs such as: - Hygiene - Food - Every day activities	In need of assistance for some daily living activities [use if client is incarcerated]	Very minimal assistance to meet daily living activities.	Meets all basic needs of daily living without assistance.	Provides beyond basic needs of daily living for self and family.
Family /Social Relations	Lack of support from: - Family - Friends	Limited Support from: - Family - Friends - Family and Friend lack the ability/skills to help	Minimal support from family/friends: Family/Friends are learning to communicate and support.	Strong support from family or friends.	Has healthy/expanding support and good communication.
Community Involvement	Not applicable due to crisis. [use if client is incarcerated]	Socially isolated No social skills Lacks motivation to become involved.	Willing to be involved in the community, but is lacking skills or not aware of available opportunities.	Some community involvement, but has barriers such as: - Transportation - Childcare	Actively involved in community.
Parenting Skills *	Safety concerns regarding parenting skills. [i.e. 300 petition filed]	Parenting skills are minimal. [i.e. non-custodial parents, supervised visits]	Parenting skills are evident, but continuous growth is rec'd [i.e. unsupervised or monitored visits, VFM, IS]	Parenting skills are adequate. [i.e. FM, joint custody, recently closed VFM/ IS case]	Parenting skills are well developed. [i.e. has insight regarding strengths and weaknesses, know how to get

					support when needed]
Legal Dependency	Child has been removed and parent is in crisis.	New referral of abuse or neglect. High risk incident: DV, relapse, loss of housing or employment.	Minimally stable and open to receiving help.	Engaged in services, creating safety plans, and actively seeking support. Child is living with or regularly visiting parent.	Has safety plan(s) in place. No current involvement in the child welfare system.
Substance Abuse	Meets criteria for severe abuse/dependence; Resulting problems are severe so that require institutional living or hospitalization may be needed.	Meets criteria for dependence: • Preoccupation with use or obtaining drugs/alcohol • Withdrawal • Behaviors demonstrate avoiding withdrawals • Use results in neglect of essential life activities.	Use within last 6 months; evidence of occupational, emotional or physical problems (such as disruptive behavior or housing problems) have persisted for at least one month.	Client has used during last 6 months, but no evidence of recurrent dangerous use.	No drug use/alcohol abuse in last 6 months. [use when no substance abuse concerns are presented]